

TobCare Summary: Core Realities & Challenges

1. Frontend is the Easy 5%

Building a React UI is trivial. The challenge lies in the backend logic, healthcare workflows, and compliance. Many teams fail because they stop at making a pretty interface.

2. Hard Part #1 — Doctor Availability Logic

Doctors in Lebanon have chaotic, unpredictable schedules:

- Multiple hospitals
- Emergencies
- Shifting clinic hours
- No reliable weekly schedule

System must support:

- Real-time availability
- Multiple calendars
- Emergency overrides
- Trustworthy slot booking

3. Hard Part #2 — Medical Records Unification

Lebanon's medical record environment is fragmented:

- Paper files, WhatsApp images, PDFs
- Mixed languages (Arabic/French/English)
- Hospitals using incompatible systems

To unify records you need:

- A clean data model (diagnoses, notes, prescriptions)
- Permission layers
- Version history
- Audit logs
- Secure storage

4. Hard Part #3 — Compliance and Privacy

Doctors require:

- Strict confidentiality
- Consent flows
- Audit logs of access
- Encryption at rest and in transit

Healthcare must feel serious and official or users won't trust it.

5. Hard Part #4 — Hospital Integration

Hospitals use Epic, Cerner, old systems, or paper.

Integrations require:

- FHIR/HL7 understanding
- Political approval
- Legacy system handling
- Long negotiation cycles

This is why existing Lebanese apps don't integrate deeply.

6. Hard Part #5 — Real Adoption by Doctors

Doctors resist switching tools. You must prove:

- Less chaos than WhatsApp
- Reliable booking
- Real benefit to their workflow

- Smooth onboarding

Product-market fit matters more than features.

Conclusion:

The frontend is easy. The real work is scheduling intelligence, record management, compliance, interoperability, trust, and doctor adoption.